

CITYCARE GENERAL HOSPITAL

EMERGENCY DEPARTMENT STANDARD
OPERATING PROCEDURE

Triage, Resuscitation & Emergency Care Protocols

SOP Code	SOP-ED-004
Department / Unit	Emergency Department (ED)
Document Version	2.1
Effective Date	01 January 2025
Review Date	31 December 2025
Prepared By	Quality Assurance & Clinical Governance Team
Approved By	Chief Medical Officer, CityHospital Group
Applicable To	All Clinical and Administrative Staff
Classification	CONFIDENTIAL — Internal Use Only

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1. PURPOSE & SCOPE

This SOP establishes the standards for emergency patient management at CityHospital Emergency Department. It covers triage, resuscitation, emergency treatment, and safe disposition of all patients presenting to the ED regardless of age, condition, or time of presentation.

- Deliver life-saving interventions within evidence-based time targets.
- Ensure systematic, reproducible triage using the Manchester Triage System (MTS).
- Coordinate multi-disciplinary emergency response effectively and efficiently.
- Manage patient flow to minimise waiting times and prevent corridor care.
- Provide compassionate, trauma-informed care in a high-pressure environment.
- Comply with all national emergency care standards and accreditation requirements.

2. DEFINITIONS & TRIAGE CATEGORIES

Level	Category	Colour	Target Physician Time	Clinical Example
1	Immediate	RED	0 minutes	Cardiac arrest, major trauma, anaphylaxis
2	Very Urgent	ORANGE	10 minutes	Chest pain, stroke, severe dyspnoea
3	Urgent	YELLOW	30 minutes	Fracture, moderate pain, altered GCS
4	Standard	GREEN	60 minutes	Minor lacerations, UTI, sprain
5	Non-Urgent	BLUE	120 minutes	Medication refill, minor rash, review

MTS — Manchester Triage System. GCS — Glasgow Coma Scale. UTI — Urinary Tract Infection.

3. ROLES & RESPONSIBILITIES

Role	Department	Responsibilities
ED Consultant	Emergency Dept.	Clinical leadership, complex decisions, major incident command
Emergency Physician	Emergency Dept.	Patient assessment, investigations, treatment, disposition
Triage Nurse	Emergency Dept.	MTS triage within 5 min, initial assessment, re-triage every 30 min
Resuscitation Nurse	Emergency Dept.	Resus bay management, airway support, defibrillation, documentation
ED Coordinator	Emergency Dept.	Bed flow, ambulance coordination, surge management
Security Officer	Security	De-escalation, restraint assistance, visitor management
Social Worker	Social Services	Safeguarding assessments, mental health support, vulnerable patients

4. TRIAGE WORKFLOW

- Step 1.** Patient arrives (walk-in or ambulance); registered at front desk or directed straight to triage bay for Level 1–2.
- Step 2.** Triage nurse performs 2-minute MTS assessment: chief complaint, pain score (0–10), vital signs, and visual survey.
- Step 3.** Triage category assigned (1–5); colour-coded wristband applied immediately.
- Step 4.** Level 1 and 2 patients immediately escorted to treatment bay or resuscitation room; no waiting.
- Step 5.** Level 3–5 patients directed to waiting area with clear ETA; monitored via re-triage every 30 minutes.
- Step 6.** Ambulance divert protocol activated when ED capacity exceeds 110%; base hospital and ambulance control notified.
- Step 7.** Parents or carers of paediatric patients kept with child throughout triage and treatment where clinically safe.
- Step 8.** Interpreter service activated immediately for patients with limited English proficiency.

WARNING: All waiting patients must be re-triaged every 30 minutes. Failure to re-triage is a patient safety risk and constitutes a reportable clinical incident.

5. RESUSCITATION & LEVEL 1 RESPONSE

5.1 Resuscitation Team Activation

- Trauma call or cardiac arrest call announced via overhead PA and bleep cascade system.
- Resuscitation team assembled within 2 minutes: ED consultant, 2 nurses, anaesthetist, runner, and scribe.
- Resuscitation bay checked and restocked at start of every shift; signed checklist completed and filed.
- Team leader clearly identified; roles assigned before patient arrives where possible.

5.2 ABCDE Primary Survey Protocol

Step 1. Airway: Assess and secure airway; suction, jaw thrust, NPA or OPA, RSI if indicated by anaesthetist.

Step 2. Breathing: High-flow oxygen applied; bag-valve-mask ventilation if inadequate; bilateral air entry confirmed; chest decompression if tension pneumothorax suspected.

Step 3. Circulation: Large-bore IV access x2; fluid resuscitation as per protocol; haemorrhage control; 12-lead ECG; defibrillation if VF or pVT confirmed.

Step 4. Disability: GCS assessment; pupil examination; blood glucose measurement; posturing assessment; seizure management if indicated.

Step 5. Exposure: Full body survey head to toe; temperature management; log-roll for posterior injuries; document all findings.

5.3 Post-Resuscitation Actions

- Stabilised patient transferred to ICU or HDU with continuous monitoring and documented handover.
- Family liaison nurse assigned to relatives; update given in private quiet room.
- Formal debrief conducted after every resuscitation event; learning points documented and shared.
- Resuscitation equipment restocked and checked within 30 minutes of resus event completion.

6. MAJOR INCIDENT & MASS CASUALTY PROTOCOL

A Major Incident (MI) is declared when ED resources are, or are likely to be, overwhelmed by a sudden influx of casualties. Declaration activates the hospital-wide Major Incident Plan.

Step 1. ED Consultant or senior nurse declares MAJOR INCIDENT; hospital switchboard activates full bleep cascade.

Step 2. All off-duty surgical, anaesthetic, and ED consultants recalled; coordination centre activated.

Step 3. START triage system applied for mass casualties: colour-coded tags applied on arrival.

Step 4. Surge areas opened: corridor care protocols, day surgery unit repurposed as overflow treatment area.

Step 5. Non-urgent patients in ED diverted or expeditiously discharged to create maximum capacity.

Step 6. Blood bank placed on massive transfusion standby; theatre team placed on emergency call.

Step 7. Hospital Incident Commander assumes overall coordination; liaises with emergency services.

Step 8. All media enquiries handled solely by the designated hospital communications officer.

7. MENTAL HEALTH EMERGENCY PROTOCOL

- Patients presenting with a psychiatric emergency assessed with full mental status examination by ED physician.
- Risk assessment for self-harm and harm to others documented using validated tool (Manchester Self-Harm Rule).
- Psychiatry on-call contacted within 30 minutes for all active suicidal ideation cases; response expected within 1 hour.
- Patient placed in designated safe room: ligature-point free, no unsupervised access to sharps or medications.
- 1:1 nursing observation assigned; reassessment documented every 15 minutes.
- Family or carer involved in safety planning with explicit patient consent.
- Mental Health Act assessment initiated if criteria for involuntary admission are met.
- Safeguarding referral made if children or vulnerable adults are identified at risk.

8. PAEDIATRIC EMERGENCY PROTOCOL

- All paediatric patients under 16 years assessed using Paediatric Triage Tape (Broselow or JCTAS).
- Weight-based medication dosing calculated using Broselow tape or the EMR paediatric dosing calculator.
- Parent or guardian present at all times unless a safeguarding concern has been identified.
- Paediatric emergency trolley stocked, checked, sealed, and signed at the start of every shift.
- Paediatric specialist or on-call paediatrician involved for all Level 1 and 2 paediatric presentations.
- Suspected child abuse: mandatory safeguarding referral; social worker and police liaison officer activated; senior physician informed.
- Paediatric resuscitation follows Paediatric Advanced Life Support (PALS) guidelines at all times.
- Child-friendly environment maintained in paediatric waiting and treatment areas; play specialist available during business hours.

9. DOCUMENTATION & HANDOVER

9.1 ED Documentation Standards

- Triage note completed within 5 minutes of triage assessment and entered in EMR.
- Physician assessment note completed within 30 minutes of seeing the patient.
- All interventions, medications, and investigations time-stamped and author-attributed in EMR.
- Disposition decision (admit, discharge, or transfer) documented with full clinical rationale.
- Medication administration: two-nurse check for high-risk drugs; EMR administration record completed immediately.

9.2 SBAR Handover to Inpatient Team

- **Situation:** Patient name, age, MRN, presenting complaint, triage category, and current clinical status.
- **Background:** Relevant medical history, current medications, allergies, and pertinent social history.
- **Assessment:** Clinical findings, investigations ordered or resulted, and working differential diagnosis.
- **Recommendation:** Immediate actions required, monitoring frequency, and clear escalation triggers.

10. QUALITY INDICATORS, EXCEPTIONS & BEST PRACTICES

10.1 Key Performance Indicators

KPI	Target	Measurement Frequency
Time to triage from arrival	100% within 15 minutes	Daily
Level 1 physician response time	100% within 0 minutes	Per event
Door-to-ECG for chest pain	Less than or equal to 10 minutes	Daily
Door-to-needle for thrombolysis	Less than or equal to 60 minutes	Per event
ED length of stay (admission or discharge)	80% within 4 hours	Daily
Re-triage compliance	100% within 30 minutes	Per shift audit

10.2 Best Practices

- Apply trauma-informed approach: assume any patient may have experienced trauma; avoid re-traumatisation in language and examination.
- Utilise point-of-care ultrasound (POCUS) to expedite diagnosis and reduce radiation exposure.
- Sepsis Six bundle initiated within 1 hour of sepsis recognition: blood cultures, IV antibiotics, IV fluids, lactate, urine output, and oxygen.
- Proactive pain management: pain assessed and treated within 20 minutes of triage for Level 2 and 3 patients.
- Regular walkrounds of waiting area by charge nurse to identify any clinically deteriorating patients.

10.3 Exceptions

- Ambulance bypass: ED Consultant only may authorise; documented in EMR; reviewed at next governance meeting.
- Police or forensic patients: dedicated assessment room; chain of evidence protocols strictly observed.
- Intoxicated patients: assessed for clinical safety; not triaged as non-urgent on basis of intoxication alone.